

Public Health Emergency Operations Center

"COUSP DRC"

MVEB-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°073/MVE B_26/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (48)	• Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Lubunga, Makiso-Kisangani and Mangobo
Reporting date	July 26, 2026
Publication date	July 27, 2026

INDICATORS KEYS — CURRENT TO JULY 26, 2026

CONFIRMED CASES — 5 PROVINCES 3,262 +62 new cases (Ituri 53 · N-Kivu 8 – Haut-Uele 1) 	CONFIRMED DEATHS · LEATHITIS 1,437* · (44.1%) +32 deaths (Ituri 23 · North Kivu* 8 - Upper Kivu) Uele 1) 	SUSPECTED CASES OF THE DAY 326 Ituri 242 · N-Kivu 81 · Haut-Uélé 3 
PATIENTS IN ISOLATION 723 Ituri 531 · North Kivu 192 	CURED — CUMULATIVE 583 +12 recovered (Ituri 11 – North Kivu 1) 	NATIONAL CONTACT TRACKING 78.2% 12,122/ 15,498 views · Ituri 75.6% - Top-Uele 60.0% - North Kivu 85.2% 

* Harmonisation of the number of deaths per health zone in the province of North Kivu (minus 2 deaths).

1. Epidemiological Highlights

- **Epidemiological situation:** No new health zones affected. The total remains at 48 affected health zones. 140 in the five provinces, of which 47 remain in active transmission.
- **Coordination:** Arrival of the joint INSP/COUSP & WHO mission team in Buta (Bas-Uélé) as part of the strengthening preparedness and response to Ebola Bundibugyo virus disease.
- **Case trends:** In the last 24 hours, 62 new confirmed cases, 32 confirmed deaths, and 12 recoveries were recorded:
 - o Ituri: 53 new confirmed cases were notified, mainly in Rwampara (18), Bunia (11), Mongbwalu (8), Nizi (7), Drodoro (4), Bambu (3), Aungba (1) and Nia-Nia (1), 23 deaths among which 60.9% were community deaths.
 - North **Kivu**: 8 new confirmed cases were reported (Butembo 3, Katwa 2, Beni 1, Kalunguta 1, and Kyondo 1), as well as 8 deaths (after harmonization with health zones, see Table 2), including 3 community deaths. The provincial total reaches 314 confirmed cases and 204 deaths, representing a case fatality rate of 65.0%.

o **Haut-Uélé** : 1 new confirmed case and 1 community death in the Pawa health zone have been reported.

The provincial total rises to 39 confirmed cases and 21 deaths, representing a case fatality rate of 53.8%, spread across 5 of the 13 health zones in the province.

o **South Kivu and Tshopo** : No new confirmed cases or deaths have been reported. The cumulative totals remain at 5 cases, 4 deaths and 1 recovery in Tshopo and 3 cases, including 1 death and 2 recoveries in South Kivu, where the Miti-Murhesa health zone has gone 61 days and the Tshopo province 12 days without a new confirmed case.

- **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 88.9% of cumulative cases and 85.5% of new daily confirmations. Contact tracing performance remains insufficient and requires strengthened surveillance activities.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it

The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 48 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.

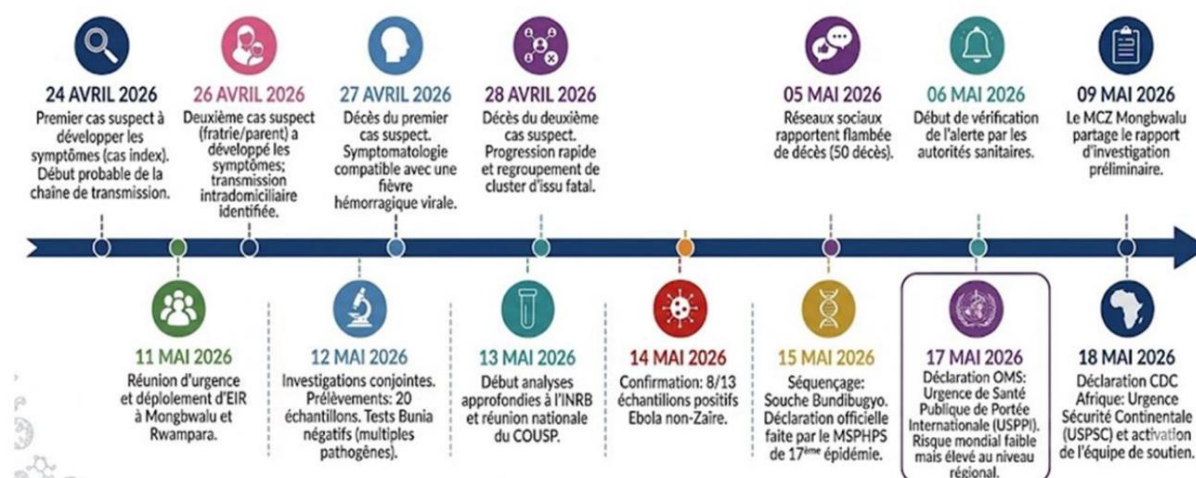


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	2,901	1,207	41.6%	28 / 36 (77.8%)	53
North Kivu*	314	204*	65.0%	11/34 (32.4%)	8
Haut-Uélé	38	21	52.6%	5/13 (38.5%)	1
Tshopo	5	4	80.0%	3/23 (13.0%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,262	1,437	44.1%	48 out of 140	62

* Harmonisation of the number of deaths per health zone in the province of North Kivu (minus 2 deaths).

The Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 88.9% of confirmed cases (n = 2,901) and 84.0% of deaths (n = 1,207), representing a case fatality rate of 41.6%. The health zones of Bunia (773 cases), Rwampara (568), Mongbwalu (506 cases), Nizi (357), Nyankunde (114), and Lita (117 cases) remain the most affected, accounting for approximately 83.9% of reported cases at the provincial level and 74.6% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2,901	1,207	41.6%	53	14	9	23
Adja	11	1	9.1%				
Aru	5	5	100.0%	0	0	1	1
Ariwara	7	1	14.3%				
Aungba	5	4	80.0%	1	0	0	0
Bamboo	57	11	19.3%	3	0	0	0
Boga	1	1	100.0%				
Bunia	773	220	28.5%	11	1	4	5
Damascus	16	8	50.0%				
Drodoro	10	5	66.7%	4	1	0	1
Fataki	17	6	35.3%				
Gethy	1	0	0.0%				
Kambala	2	0	0.0%				
Kilo	23	11	47.8%				
Komanda	35	24	68.6%				
Lita	117	66	56.4%				
Logo	9	3	33.3%				
Lolwa	4	1	25.0%				
Mahagi	3	1	33.3%				
Mambasa	9	4	44.4%				
Mandima	13	9	69.2%				
Mangala	100	64	64.0%				
Mongbwalu	506	244	48.2%	8	5	2	7
Nia Nia	90	47	52.2%	1	1	0	1
Nizi	357	171	47.9%	7	5	0	5
Nyankunde	114	31	27.2%				
Rimba	8	4	50.0%				
Rwampara	568	241	42.4%	18	1	2	3
Tchomia	40	24	60.0%				
North Kivu*	314	204*	65.0%	8	3	5	8
Blessed	44	34	69.8%	1	4	0	4
Butembo	73	49	67.1%	3	1	0	1
Goma	1	0	0.0%				
Kalunguta	5	3	75.0%	1	0	0	0
Katwa	140	88	62.9%	2	-2*	0	-2
Kyondo	12	7	58.3%	1	0	0	0

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Mabalako	1	0	0.0%				
Masereka	4	0	0.0%				
Musienene	29	19	65.5%	0	0	5	5
Oicha	4	3	75.0%				
Vuhovi	1	1	100.0%				
South Kivu	3	1	33.3%	—			—
Miti-Murhesa	3	1	33.3%				
Haut-Uélé	39	21	53.8%	1	1	0	1
Boma Mangbetu	7	4	57.1%				
Isiro	12	7	58.3%				
Pawa	8	6	75.0%	1	1	0	1
Rungu	1	0	0.0%				
Wamba	11	4	36.4%				
Tshopo	5	4	80.0%	—			—
Lubunga	1	0	0.0%				
Makiso-Kisangani	3	3	100.0%				
Mangobo	1	1	100.0%				
Total	3,262	1,437	44.1% 62		18	14	32

*

Harmonisation of the number of deaths per health zone in the province of North Kivu (minus 2 deaths).

2.3 Mapping of confirmed cases

The spatial distribution confirms that transmission is mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Nyankunde and Lita, with transmission continuing in already affected areas, including the Rungu health zone in Haut-Uélé.

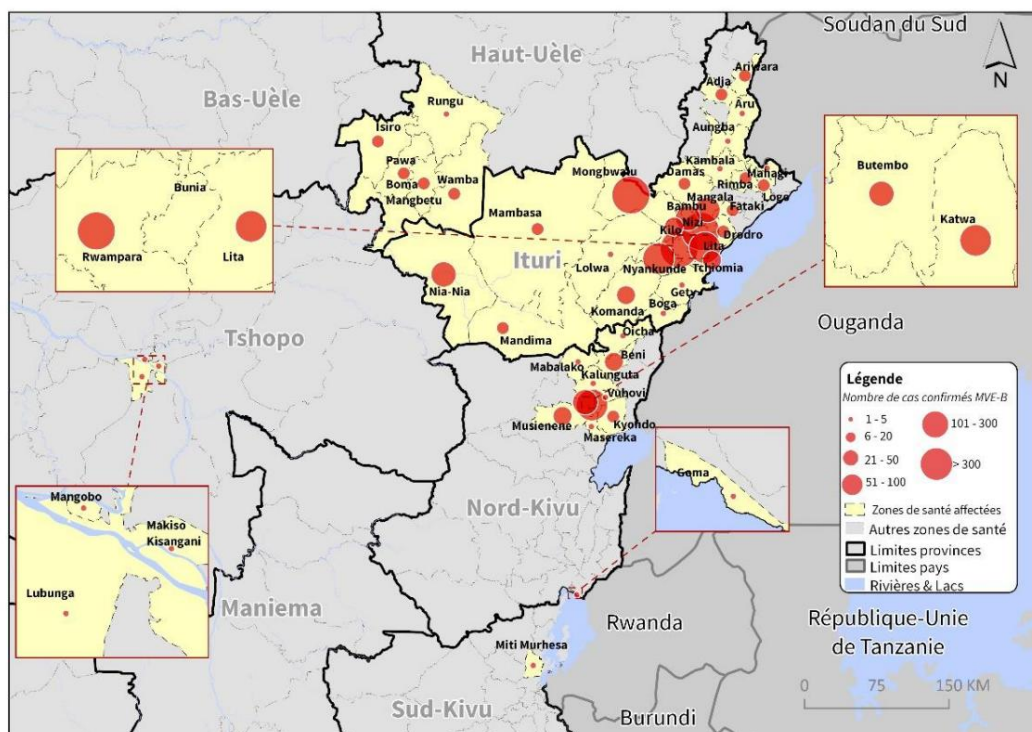
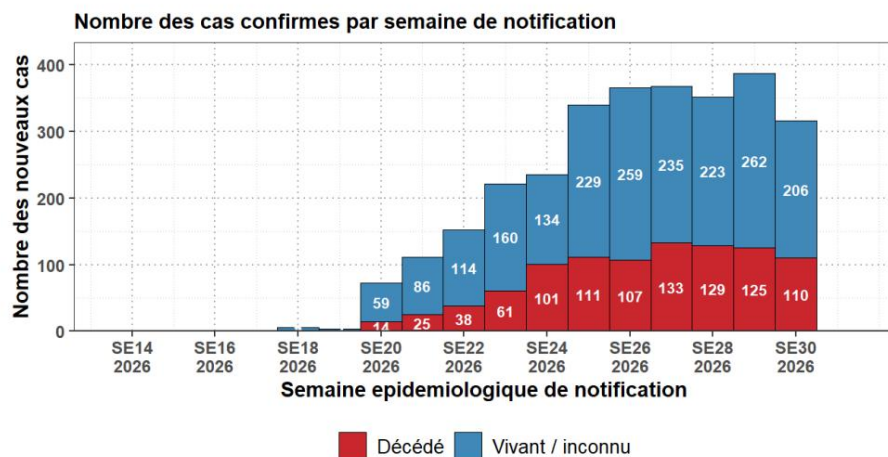


FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

The continued transmission in the Rungu health zone in Haut-Uélé demonstrates the continued transmission in already affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S25 to S30, with fluctuations related to notification delays and the gradual consolidation of data.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de notification renseignés, n = 2 931.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

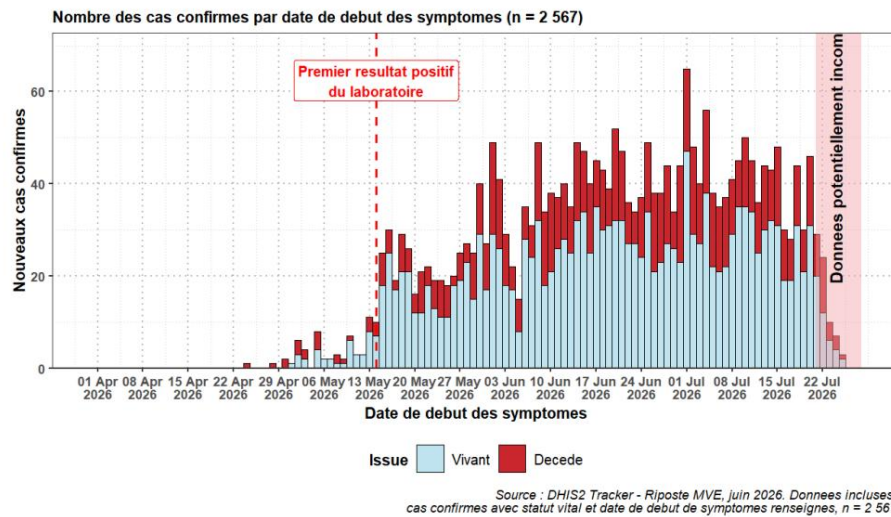


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

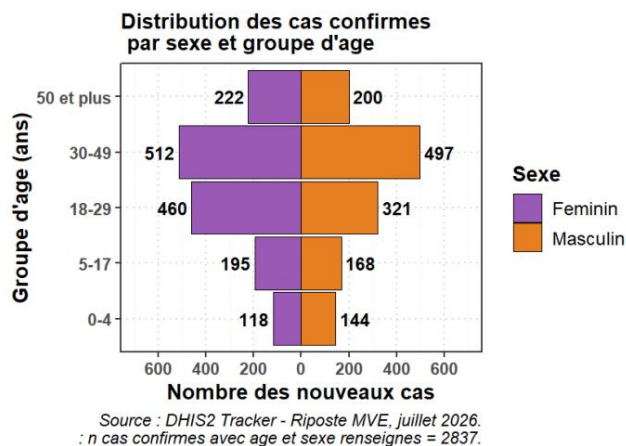
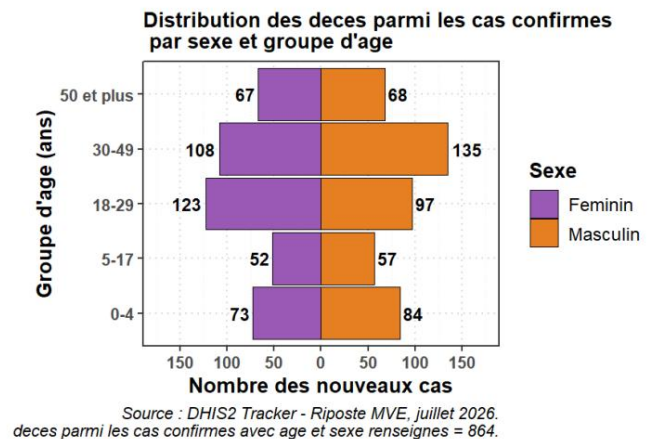


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	6	36	59	ND	ND	101
New alerts received	587	614	1	ND	ND	1202
Total alerts for the day	593	650	60	ND	ND	1303
Alerts investigated	482	596	0	ND	ND	1078
Investigation rate (24 h)	81.3%	91.7%	0.0%	ND	ND	82.7%
Validated alerts — alive Validated	168	46	2	ND	ND	216
alerts — deceased (comm.)	74	35	1	ND	ND	110
Total suspected cases for today	242	81	3	ND	ND	326

In total, 1,303 alerts (including 1,202 new ones) were received, with 1,078 investigated within 24 hours (82.7%) and 326 confirmed (216 active cases and 110 community deaths). This improvement reflects a reduction in investigation delays and the consolidation of data from the affected provinces. The improved investigation rate is linked to the reduction in investigation delays and the strengthening of surveillance teams in the affected provinces.

3.2 Laboratory: tests and positivity

Ÿ **Ituri** : 186 samples documented in the collection network, of which 53 were positive (39 living and 14 deceased), i.e., a positivity rate apparent of 28.4%.

Ÿ **North Kivu** : 113 samples received and analyzed, of which 8 were positive (5 living and 3 deceased), representing a positivity rate of 7.0%.

Ÿ **Haut-Uélé** : 8 samples received and 8 analyzed, among which 1 was positive (deceased), i.e. a positivity rate of 12.5%.

Ÿ **Tshopo** : 5 samples received and analyzed. All of these samples came back negative after analysis.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri*	10,901	8,245	75.6%
North Kivu*	4,447	3,787	85.2%
Tshopo	ND	ND	ND
Haut-Uélé	150	90	60.0%
South Kivu (end of monitoring)	0	0	N / A
Total	15,498	12,122	78.2%

* In Ituri, the monitoring rate was calculated only for health zones that had submitted reports. The health zones of Damas, Fataki, Kilo, Kambala, Logo, Mambasa, Nyankunde and Rimba did not have data available at the time of report consolidation.

Contact tracing remains insufficient at the national level (78.2%), below the operational threshold of 95%. Ituri only managed to follow up on 8,245 out of 10,901 contacts (75.6%). North Kivu achieved a performance of 3,787 out of 4,447 contacts. (85.2%). Haut-Uélé only followed up with 90 out of 150 contacts (60.0%). Tshopo maintains a relatively high performance with a 90.9% follow-up rate. Follow-up is closed in South Kivu.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 5 alerts notified

Five (5) alerts, all active, were intercepted at the PoE/PoC and all validated: two at PoC Malaba, one at PoC Makoko 2, one at PoC Foner Mambasa, and one at PoC Odhumoni. Only one was referred to the Foner Mambasa CTE.

North Kivu — 0 alerts notified

No alerts were notified to the province's 16 activated PoE/PoCs, whose completeness and promptness of reporting reach.

Tshopo — 0 alerts notified

No alerts were notified to the various PoE/PoC devices activated in the province.

Haut-Uele — 0 alerts notified

No alerts were notified to the various PoE/PoC devices activated in the province.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri	North Kivu	Tshopo	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (16/16)	60%	100% (76/76)
Number of people who have switched to PoE/PoC	88,216	54,718	9105	152039
% of travelers screened	93.6%	96.8%	100%	96.8%
% of travelers who washed their hands	91.7%	96.8%	100%	93.7%
% of travelers made aware	94.9%	96.8%	100%	95.6%
Number of alerts notified	5	0	0	5
Number of validated live alerts	5	0	0	5
Number of bodies recovered today	0	0	0	0
Number of problematic contacts intercepted today	0	0	0	0
% of validated live alerts referred to CT/CTE % of	20.0% (1/5)	N/A	0	20.0% (1/5)
lifeless bodies swabbed	N/A	N/A	0	N/A
Number of positive results of referred suspected cases	1	0	0	1

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator		Ituri	North Kivu Haut-Uele		Tshopo Together	
Number of beds		800	141	ND	ND	941
Patients in bed (Day -1)		498	183	ND	ND	681
Total admissions (24 hours)		96	30	ND	ND	126
Total admissions		594	213	ND	ND	807
Exits	Deceased	11	3	ND	ND	14
	No case	40	17	ND	ND	57
	Healed	11	1	ND	ND	12
	Escapees	1	0	ND	ND	1
	Transferred to the HGR	0	0	ND	ND	0
Total outflows (24 h)		63	21	ND	ND	84
Patients in isolation (end of day),		531	192	ND	ND	723
including confirmed cases		304	21	ND	ND	325
including suspects		227	171	ND	ND	398
Occupancy rate		66.4%	136.2%	ND	ND	76.8%

In total, 723 patients were in isolation, including 325 confirmed cases and 398 suspected cases, for an overall occupancy rate of 76.8 % (723 patients for 941 beds), with 807 admissions and 84 discharges (14 recovered, 14 deceased, 57 non-cases, and 1 escaped). Ituri bears the brunt of the burden with 531 patients in isolation (304 confirmed, 227 suspected) out of 800 beds, representing a 66.4% occupancy rate. In North Kivu, 192 patients are in isolation (21 confirmed, 171 suspected) for an occupancy rate of 136.2% (141 beds). Haut-Uele and Tshopo did not provide data at the time of compilation.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	24 (85.7%)	1 (100%)
Confirmed cases are being monitored.	95 (67.4%)	6 (100%)
New suspected cases	30 (45.5%)	1 (100%)
Suspected cases are being monitored	156 (95.1%)	8 (100%)
Laboratory results announced (including positive ones)	62 (20)	7 (2)
Children separated — nursery / community	1/0	0/0
CTE support staff	197	5
Frontline Personnel (PPL)	32	5
Members of households and communities	40	0
EDS supported (SMSPS)	7	0

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVI

Health Zone	Confirmed cases	Cured. In hospital.	Death	
Bunia	35	18	2	15
Rwampara	33	24	2	7
Mongbwalu	16	6	2	8
Nyankunde	14	11	1	2
Nia-Nia	10	3	7	0
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Logo	1	0	1	0
Total — lethality 30.1%	123	67	19	37

PPL = healthcare workers and frontline providers.

In North Kivu, 11 cases of people with disabilities have been confirmed, including 3 deaths. To date, we have recorded a total of 134 confirmed cases of people with disabilities and 40 deaths.

7 Pillar-Based Response Activities

COORDINATION Holding the coordination meeting with the various partners; Monitoring of field activities in the health zones of Rwampara and Mambasa; Briefing of the members of the provincial coordination team in Buta, comprising the technical services managers of the ministries of human, animal, and environmental health; Consolidation of daily reports from the affected provinces; Arrival of the joint WHO, INSP, and COUSP mission with a large shipment of IPC inputs, medicines, equipment, and other consumables in Buta, Bas Uele, as part of the support for Ebola preparedness activities; Briefing of the members of the Buta provincial coordination team, comprising technical staff from the Ministries of Human, Animal, and Environmental Health, INSP/COUSP, and the WHO, on preparedness for and response to sexual abuse, exploitation, and harassment within the framework of implementation Activities related to preparedness for Ebola Virus Disease (EVD); Briefing of 40 health workers from the Bas-Uélé Provincial General Referral Hospital on IPC, PEC, laboratory procedures, and PRSEAH (Regional Program for the Prevention of Ebola Virus Disease) as part of strengthening protection measures against EVD; Official handover of IPC inputs, medicines, equipment, and other consumables to the Governor of Bas-Uélé Province, accompanied by the President of the Provincial Assembly, the Ministers of Health and Education, as well as technical staff from the Ministry of Public Health, Population and Social Protection (MSPHPS), Fisheries and Livestock, and the Environment, health workers, and representatives from the National Institute of Public Health (INSP). COUSP and WHO in Buta.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: Continued assessment of needs PCI inputs within the framework of continuity of essential services and care; Analysis of indicators derived from DHIS2 (ongoing); Development and Budgeting of the CSSE pillar plan.
LOGISTICS ÿ Ituri: Delivery of 87 packages of PCI kits, i.e. 666 kg, to the CTE of the HGR of Bunia; 38 vehicles and 12 ambulances made available to the pillars and health zones; continuation of construction work on the CTE of Rwankole, where the installation of beds is underway). ÿ North Kivu: Delivery of 200 cartons of Plumpy'Nut to the Mutwanga Health Zone with logistical support from the WFP and deployment of service providers from various pillars in the health zones. Only 4 of the 11 health zone hotspots have submitted their logistical data, representing 36%, and only 2 out of 30 ambulances are operational.	PCI / WASH ÿ Ituri: 39 rings opened around cases out of 77 expected (50%), 42 community health workers (CHWs) monitored and supported, 10 CHWs and 25 means of transport decontaminated, 45 households of cases decontaminated within 48 hours out of 94 (47.8%), and 90 community health workers trained (Tchomia 50, Bambu 27, and Drodoro 13). All 51 identified sites have been decontaminated. Five new cases of community health workers were reported in Rwampara (2), Nia-Nia (2), and Mahagi (1), and reporting completeness remains low, at 9 out of 29 health zones. ÿ North Kivu: Continued PCI monitoring in social and solidarity economy enterprises and actions to reduce the risk of exposure of service providers. ÿ Tshopo : Training of 30 hygiene specialists from the Buta General Referral Hospital AND simulation on hand hygiene practices, the use of PPE, bio-cleaning, and the preparation of chlorine solutions within the framework of the

	Ebola epidemic preparation Bundibugyo, Buta Province Bas Uele.
COMMUNICATION (RCCE / CREC) ÿ Ituri: 459 people reached during home visits, 380 during educational talks and focus groups, 311 during community dialogues, 4,881 during mass awareness campaigns and 391 in churches, mosques and markets; 98 community alerts reported (91 alive and 7 dead) and 2 resistances lifted in Rwampara; workshop for the development and validation of the provincial CREC plan in Bunia and handover of 132 vests to the RECOs of the 11 health areas of Nia-Nia with the support of Village Reach. ÿ North Kivu: 10,522 households visited (83%), 25,409 people reached door-to-door (92%), and 886 in public places; 130 alerts of suspected cases reported by community actors (57.7% of the target), and 32 out of 34 community radio stations broadcasting at least one message. Mass communications were conducted at the CECA 20 Nazareth church in Beni, reaching 432 people, and among members of the Catholic parish in Mabuku, Kalunguta. ÿHaut-Uélé: Educational talk with the pastor of the APHC church (Pentecostal Assembly Heir of Christ, locally known as BIMA of the spirit in Isiro) Educational talk with the young musicians of the SOMANA church in Isiro; Educational talk with the young boys in the office automation program at the Mendambo Commune; Talk with 12 motorcycle taxi drivers in Wamba on prevention measures.	HOLISTIC CARE (HCC) ÿ Ituri: 11 new recoveries, including a five-week-old child orphaned of father and mother placed in temporary foster care in Mongbwalu; 531 patients in isolation, 96 new admissions and 63 discharges including 11 deaths; 962 people received hot meals and 22 ambulance trips evacuated 28 patients. ÿ North Kivu: 30 new admissions and 21 discharges (1 recovered, 3 deaths, and 17 non-cases); 192 patients remained hospitalized. The provincial total reached 130 cases treated in Ebola Treatment Centers (ETCs), including 43 deaths, representing an in-facility mortality rate of 33%. ÿ Tshopo: No new admissions; 4 non-cases discharged and no patients hospitalized. Experimental treatment continues for 2 contacts (Day 8) at the FEDI Health Center in Mangobo.
RESEARCH, INNOVATION & VACCINATION ÿ Ituri: Pillar meeting with the Incident Manager and Africa CDC for the presentation of the research program; continuation of data collection for the CAP study in the health zones of Rwampara, Bunia, Bambu-mines, Kilo, Lita, Gethy and Komanda with the support of CRS; continuation of the Partners clinical trials on the vaccine and EBO-PEP.	Dignified and secure burials (DSM) ÿ Ituri: 78 death alerts recorded, including 67 today and 11 carried over from the previous day; 61 death assessments (DAS) completed (78%); and 47 bodies recovered, including 38 in the community and 9 in specialized care facilities (ESS), across 13 out of 32 health zones. Six community refusals were recorded in Nyankunde (3), Rwampara (1), Mandima (1), and Mongbwalu (1). The cumulative total since May 20th has reached 2,708 alerts and 2,192 DAS completed. ÿ North Kivu: Support from EDS teams escorting the burial of a confirmed case at the Paidia cemetery; a public order disturbance that occurred during the burial was controlled by security forces.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ÿ Ituri: 25 stakeholders briefed and signed the code of conduct (5 women and 20 men), bringing the total to 2,490 people in 5 health zones, and 159 people sensitized in the community (103 women and girls, 56 men)	

and boys), for a total of 15,176 people. Revitalization of the 12 CBCMs in Mongbwalu and community consultations on risk factors in the village of Ndenge, in the Rwampara Health Zone.

ŸHaut-Uélé: Briefing of 40 health workers from the Bas Uele province general reference hospital on IPC, PEC, laboratory, PRSEAH as part of strengthening protection measures against BVD in the hospital meeting room; The target is 178 hospital staff: doctors, nurses, midwives, lab technicians, ambulance drivers and hygienists.

8 Main challenges, impacts and actions required

Challenge No.	Impact	Actions required	
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams	
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of governor military of Ituri)	Interruption of response activities, including dignified and safe burials Regularize payments to service providers; secure dedicated funding	
3	Insufficient capacity of CTE/CT: occupancy of 136.2% in North Kivu (bed capacity: 141) and 66.4% in Ituri (531 patients for 800 beds);	Admission delays, increased risk of nosocomial transmission Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.	
4	Contact tracking performance low in Ituri 75.6%; North-Kivu 85.2% and Haut-Uele 60.0	Uninterrupted transmission chains Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri	
5	Ambulance shortages and pre-positioning at points of control	Delays in transfer and isolation Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS	
6	Insufficient supply of MEG and medical inputs	Weakening of overall care Advocacy and urgent supply of essential medicines	
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (175/1170, i.e. 15%)	high infection rates Nosocomial risk (123 cases of PPL, including 37 deaths in Ituri and 11 PPL (including 3 deaths in North Kivu)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Butembo and Mambasa)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortcomings of the resources	Limitation of the implementation of the pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Official handover of a large batch of inputs to the province of Bas-Uélé (INSP/COUSP & WHO).



Briefing of members of the Bas-Uélé provincial coordination on general information about Ebola virus disease in Buta (INSP/COUSP & WHO)

FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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With technical support from the World Health Organization (WHO) and
Africa CDC